HEALTHATHON 2026

OncoFollow

Risk-prioritised, data-backed follow-up care for cancer patients.

Team

Venkatesh R · Naveen Krishnaa · Kiran Shinde

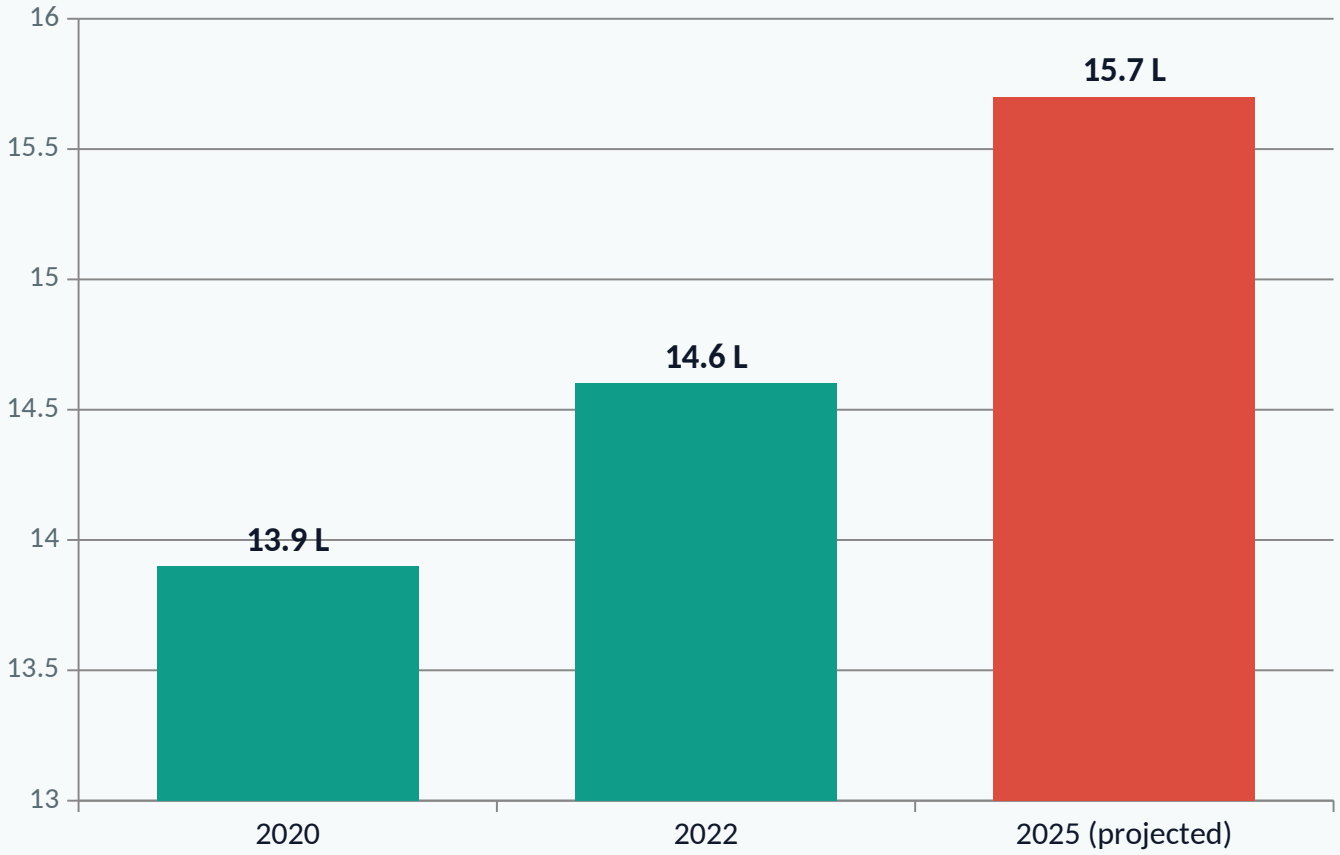
Kiran Shinde - Doctor Partner, Intern at Sir H. N. Reliance Foundation Hospital & Research Centre

Every statistic in this deck is cited at the source below its slide.

THE PROBLEM · 01

India's cancer burden is rising fast

Estimated new cancer cases, National Cancer Registry Programme (ICMR-NCDIR)



+12.8%

rise in incidence, 2020 -> 2025

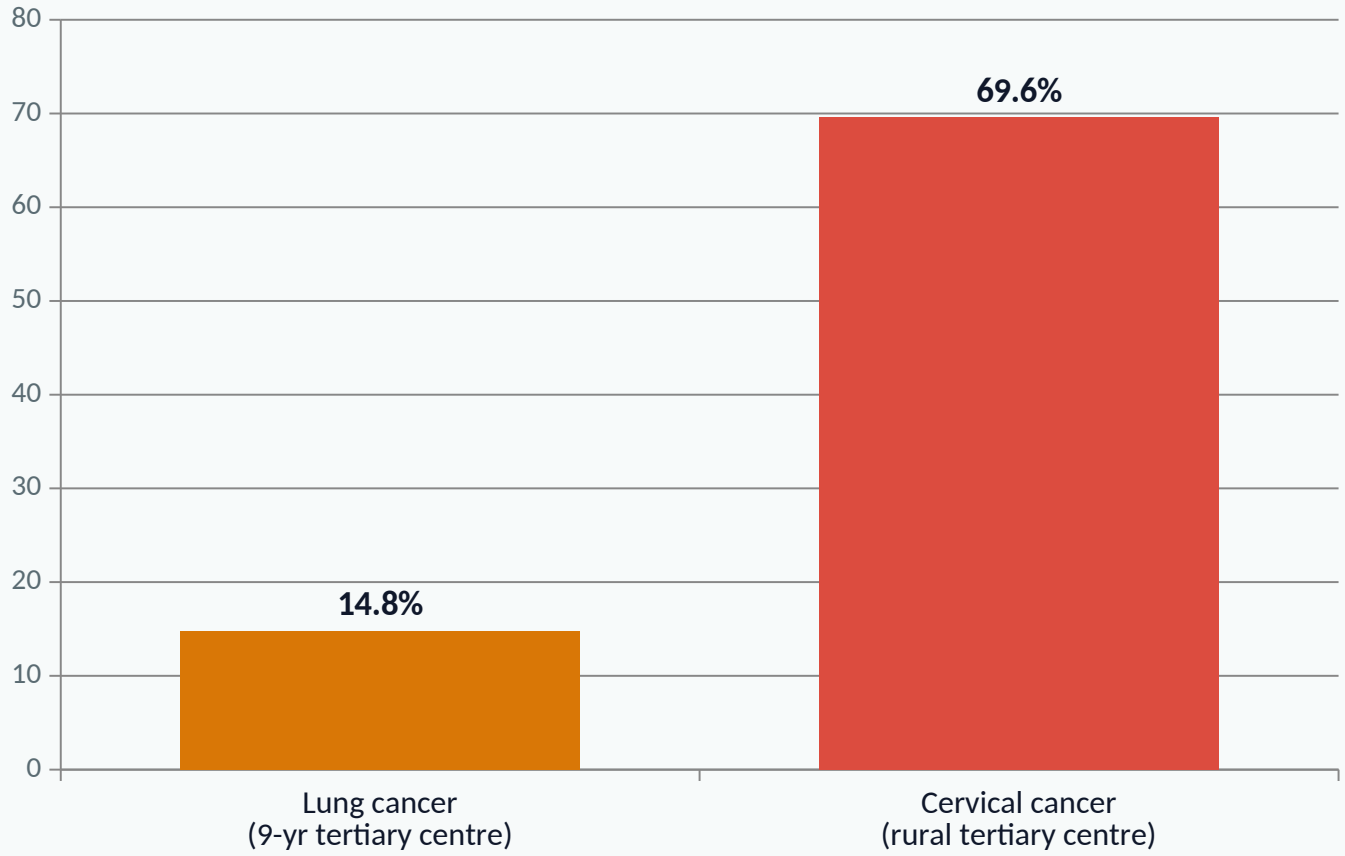
More patients means more follow-ups to track, over more years - the exact workload no manual system scales to.

Source: Cancer incidence estimates for 2022 & projection for 2025, National Cancer Registry Programme, Indian Journal of Medical Research (ICMR-NCDIR).

THE PROBLEM · 02

Patients fall out of the follow-up system

Loss-to-follow-up rate, published India cancer-centre studies



69.6%

of cervical cancer patients never returned for a follow-up visit

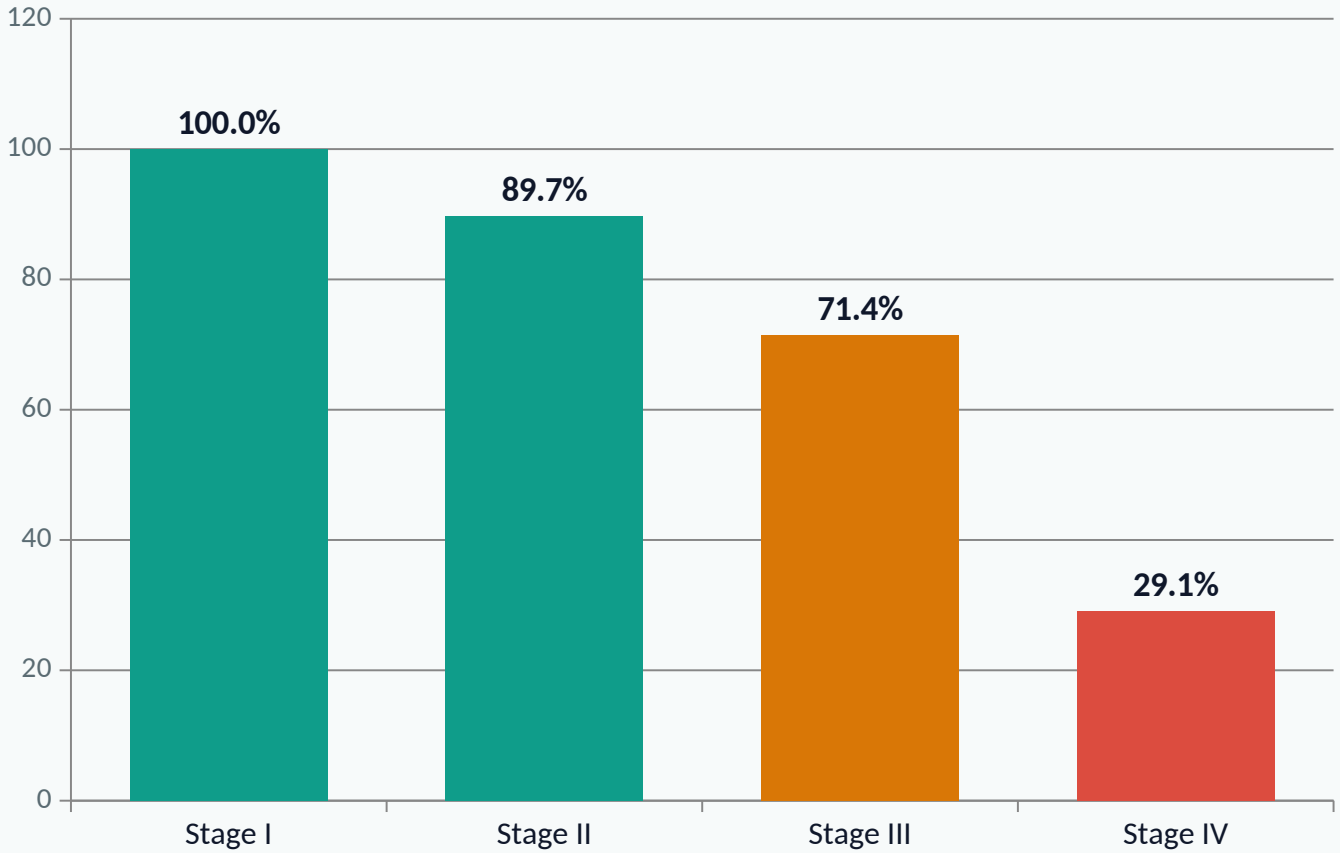
Rates vary sharply by cancer type and setting - but every study agrees: without active tracking, follow-up compliance collapses.

Source: Lung: Noronha et al., JCO Global Oncology (9-yr tertiary centre, India). Cervical: Indian Journal of Public Health, rural tertiary cancer centre, Kerala.

WHY IT MATTERS · 03

Stage at detection decides survival

5-year relative survival by stage at diagnosis, breast cancer (SEER)



91% vs 7%

colorectal cancer 5-yr survival: localized vs. distant-stage at diagnosis

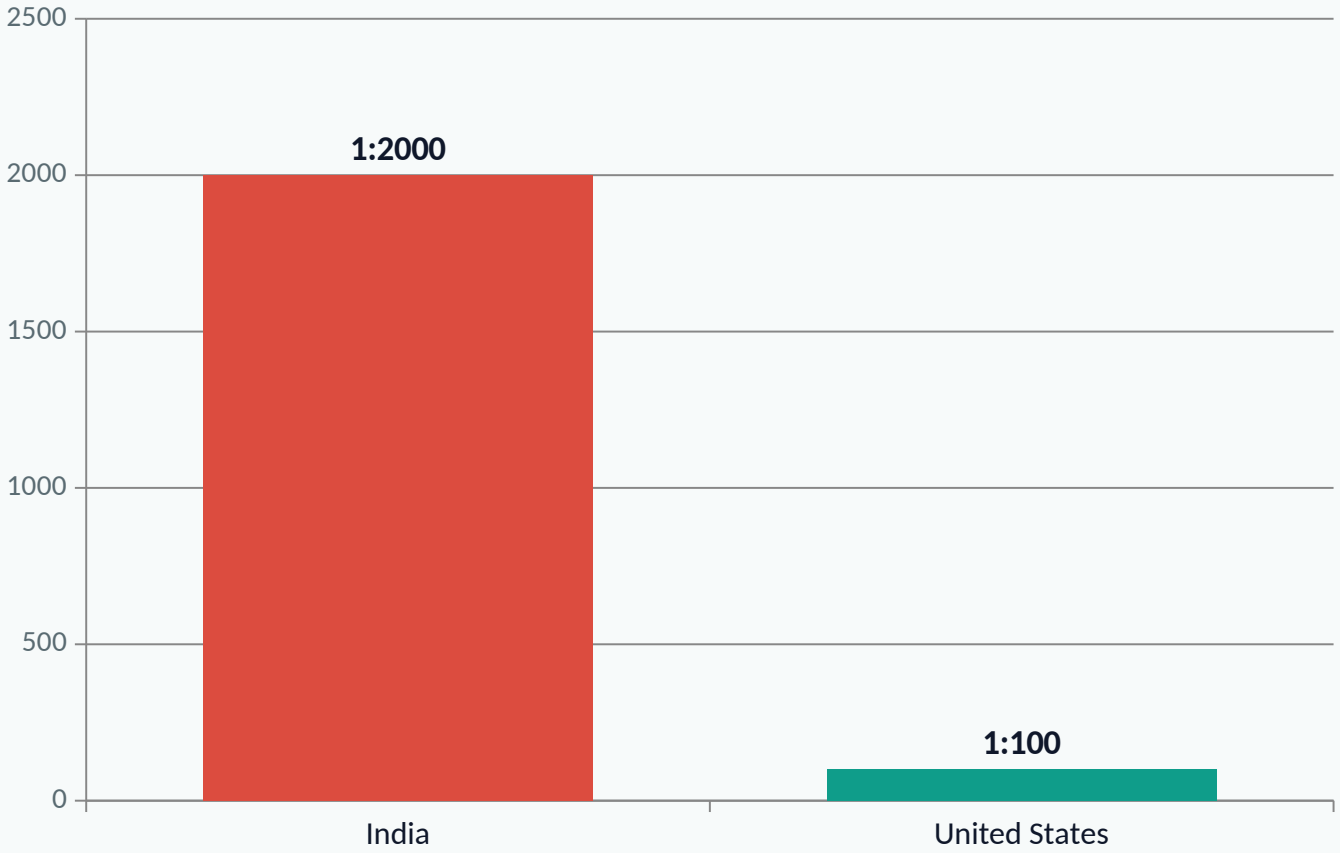
Only 37% of colorectal cancers are caught at the localized stage today.

This is the entire premise of surveillance follow-up: catching recurrence or progression while stage - and survival odds - can still be changed.

Source: SEER Cancer Stat Facts (National Cancer Institute); American Cancer Society, Breast Cancer Survival Rates.

The system doesn't have spare capacity

Cancer patients per practising oncologist



~1,000

trained oncologists nationwide, India

2,000 -> 5,000+ -> 10,000+

supply (2022) vs. demand (2022) vs. projected demand (2030)

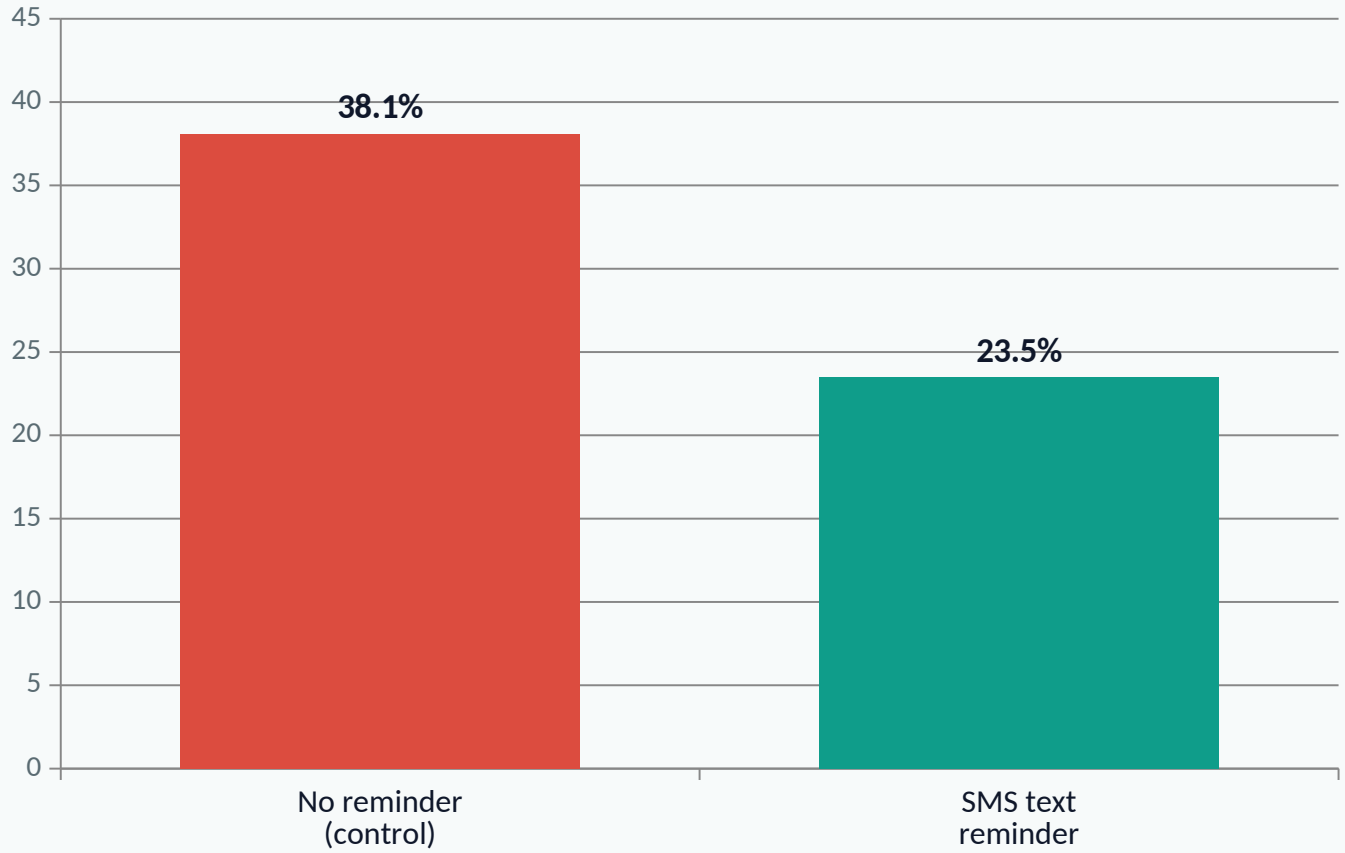
Indian oncologists already see 475 consults/year on average - more than peers in comparable economies. Manual tracking doesn't scale here; software has to.

Source: Business Standard / Statista, oncology workforce estimates; Pramesh et al., "Medical oncology in India: workload, infrastructure, and delivery of care," Indian J Med Paediatr Oncol.

THE EVIDENCE · 05

Automated reminders measurably work

Missed-appointment (no-show) rate, randomized controlled trial



-38%

relative reduction in missed appointments from a text reminder alone

OncoFollow automates exactly this mechanism for cancer follow-up specifically - triggered by clinical schedule, drafted from the doctor's own notes, not a generic template.

Source: Chen et al., "Text Message Reminders Increase Appointment Adherence in a Pediatric Clinic: A Randomized Controlled Trial," PMC5227159.

OUR SOLUTION · 06

OncoFollow

Analyses each patient's treatment record to flag who's due or overdue, ranks them by clinical urgency, and drafts a personalised reminder from the doctor's own notes - not a generic message.

1

Analyse

Reads each patient's treatment phase and cancer-specific surveillance cadence to determine what's due.

2

Prioritise

Ranks overdue patients by clinical urgency - not just days elapsed.

3

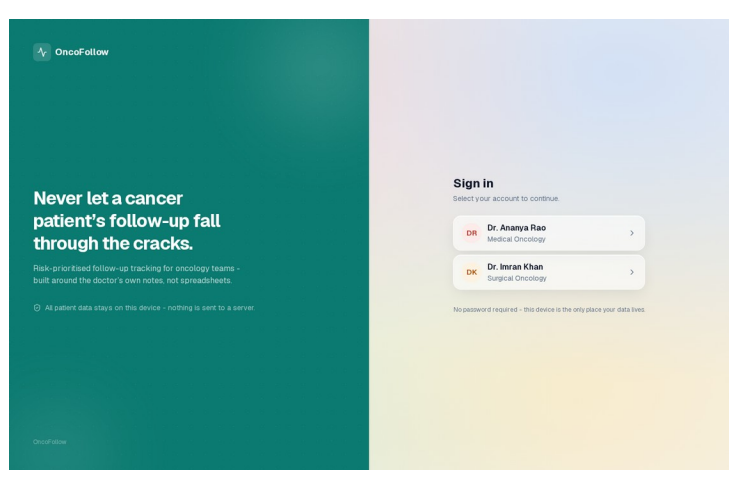
Remind

Drafts and sends a reminder grounded in that patient's own notes and symptom history.

Local-only: every byte of patient data stays on the doctor's own device - nothing is sent to a server we or anyone else controls.

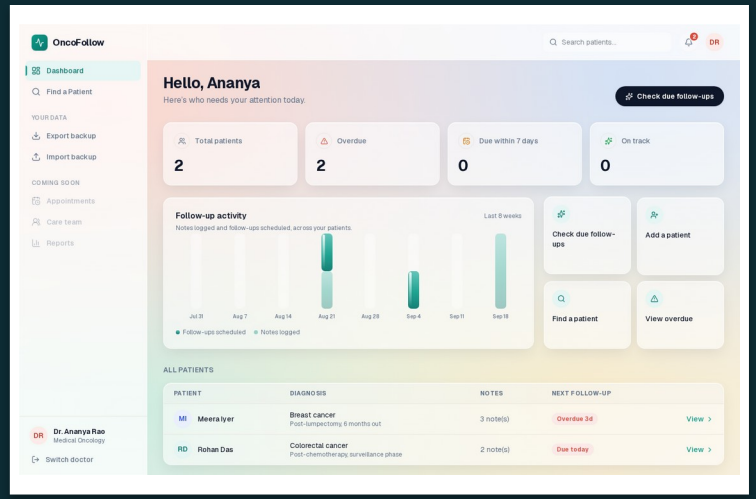
PRODUCT WALKTHROUGH · 07

From sign-in to a reminder sent



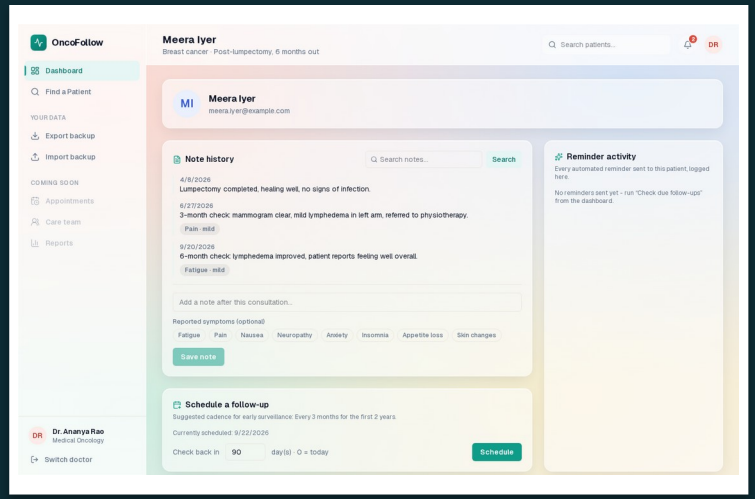
1. Sign in

Pick your account - no password, no server. Data lives only on this device.



2. Prioritised dashboard

Patients ranked by follow-up urgency, not who called first.



3. Patient record

Notes, symptoms, suggested surveillance cadence, and the reminder log - together.

KEY FEATURES • 08

Built around real oncology follow-up practice

Survivorship cadence engine

Suggests surveillance intervals from treatment phase - active, early, extended, long-term - based on common oncology follow-up cadences.

Symptom tracking

Structured, severity-tagged symptom logging at each visit - fatigue, pain, neuropathy and more - visible on the note timeline.

Risk-ranked dashboard

Patients sorted by follow-up urgency, not alphabetically or by who called - overdue and high-risk surface first.

Local-only data

Every record lives in the browser's own storage on the doctor's device. No server, no cloud database, no third party ever holds patient data.

Export / import backup

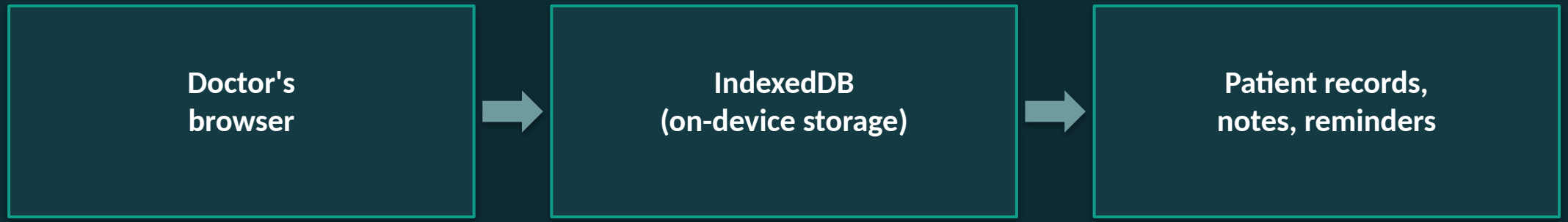
One-click backup to a file the doctor controls - the safety net for a local-only system, without reintroducing a server.

Automated, grounded reminders

Reminders are drafted from the patient's own notes, not a generic template - and logged, so every message sent is auditable.

ARCHITECTURE · 09

We are structurally unable to leak patient data



There is no fourth box - no server, no cloud database, no vendor with access.

Under India's Digital Personal Data Protection Act, 2023, an entity that never collects personal data carries none of a data fiduciary's storage, breach-notification, or consent-management burden - by construction, not by policy.

Source: Digital Personal Data Protection Act, 2023 (Government of India).

TEAM • 10

Who's building this



Venkatesh R

Engineering

Transformers & LLM serving, RLHF / alignment, ZK-SNARKs, distributed inference. Architected Laminaa's chain-agnostic agent across 10 networks. 8+ hackathon wins, ₹30L+ in prizes.



Naveen Krishnaa

Engineering

Production data engineering - ETL, Kafka, Airflow, Elasticsearch - with RAG / GenAI and cloud (Azure, GCP). B.Tech AI & ML, CGPA 9.04. HimaShield GLOF system, awarded ₹5L by MeitY.



Kiran Shinde

Doctor Partner

Intern, Sir H. N. Reliance Foundation Hospital & Research Centre. Grounds the product in real oncology follow-up workflows and clinical review of survivorship cadence logic.

From reactive care to proactive follow-up

- Care-team support - more than one doctor per patient, with role-based visibility
- Cadence rules validated against published oncology surveillance guidelines per cancer type
- Two-way reminders - a patient can reply, not just receive
- Optional, doctor-controlled encrypted sync between their own devices - still zero third-party access

REFERENCES

Sources

- ICMR-NCDIR, National Cancer Registry Programme - "Cancer incidence estimates for 2022 & projection for 2025," Indian Journal of Medical Research (IJMR).
- Noronha V. et al., "Outcomes in Lung Cancer: 9-Year Experience From a Tertiary Cancer Center in India," JCO Global Oncology, American Society of Clinical Oncology.
- "Factors Associated with Survival and Lost to Follow-up of Cervical Cancer Patients in a Tertiary Cancer Centre in Rural Kerala," Indian Journal of Public Health.
- SEER Cancer Stat Facts, National Cancer Institute (seer.cancer.gov) - Breast and Colorectal Cancer.
- American Cancer Society, "Survival Rates for Breast Cancer."
- Pramesh C.S. et al., "Medical Oncology in India: Workload, Infrastructure, and Delivery of Care," Indian Journal of Medical and Paediatric Oncology.
- Business Standard / Statista, oncology workforce demand-supply estimates, India, 2022-2030.
- Chen, et al., "Text Message Reminders Increase Appointment Adherence in a Pediatric Clinic: A Randomized Controlled Trial," PMC5227159.
- Digital Personal Data Protection Act, 2023, Government of India (Ministry of Electronics and Information Technology).

All product screenshots are of OncoFollow itself, captured directly from the running application.